WORLD HEALTH ORGANIZATION

Model United Nations
Conference - Global

FROM THE COMMITTEE BUREAU

Honourable Delegates,

The World Health Organization bureau welcomes delegates to MUNCGLOBAL's flagship conference in Ghana, MUNC-GH.

This Background Guide outlines the One-Health challenge of antimicrobial resistance (AMR)and provides the evidence base and policy options delegates will need to design global surveillance, stewardship, and R&D strategies. AMR occurs when bacteria, viruses or other pathogens no longer respond to medicines, rendering common infections deadly. This crisis knows no borders and threatens to undermine modern medicine. Please read the guide thoroughly to prepare science-based interventions that consider human, animal and environmental health. For questions, the committee bureau is available to assist. We look forward to your informed debate and practical recommendations.

The Bureau emphasizes a One Health approach. Policies must span human medicine, animal health, agriculture and the environment. Our committee must propose global strategies from surveillance and stewardship to innovation in new drugs to slow and reverse AMR trends. The bureau urges you to consider practical measures, such as antibiotic regulation, better sanitation, and support for research, to protect global health and development.

TOPIC BACKGROUND

Antimicrobial resistance (AMR) is the ability of microbes to withstand treatments that once cured them. When bacteria or viruses evolve resistance, antibiotics and antivirals become ineffective. This threat grows with **overuse and misuse of drugs** in humans, animals, and agriculture. Key facts: in 2019 AMR directly caused **1.27 million deaths** worldwide and was a contributing factor in nearly 5 million deaths. AMR erodes decades of medical progress: procedures like surgery, chemotherapy and childbirth all rely on effective antimicrobials. Without action, routine infections could again become lethal.Beyond health, AMR has grave economic consequences. WHO cites World Bank estimates that by 2050 AMR could add **$1 trillion in annual healthcare costs** and cause up to **$3.4 trillion per year in GDP losses by 2030.**

This impacts economies through lost productivity and expensive treatments. Low- and middle-income countries bear the brunt: they have fewer resources for sanitation, healthcare, and new drugs. Poverty, overcrowding, and lack of clean water accelerate drug-resistant infections. For example, lack of clean water can cause infections that are then improperly treated with antibiotics, fueling resistance. Existing global efforts include the **WHO's Global Action Plan on AMR (2015),** adopted at the World Health Assembly. This plan urges countries to develop national AMR action plans using a One Health approach (integrating human, animal and environmental health). Many countries have since created action plans, but gaps remain in implementation.

Key areas needing attention are:

(1) **Surveillance** – better tracking of AMR patterns globally;
(2) **Stewardship** – reducing unnecessary antibiotic use in people and livestock;
(3) **Infection prevention** – improving hygiene, sanitation and vaccination to reduce disease incidence;
(4) **Research & Development** – incentivizing new antibiotics, vaccines and diagnostics. Innovative financing (like public-private partnerships or prizes) may be needed to encourage pharmaceutical R&D, since new drugs for resistant bacteria have been in short supply.
(3) **Infection prevention** – improving hygiene, sanitation and vaccination to reduce disease incidence;
(4) **Research & Development** – incentivizing new antibiotics, vaccines and diagnostics. Innovative financing (like public-private partnerships or prizes) may be needed to encourage pharmaceutical R&D, since new drugs for resistant bacteria have been in short supply.

Delegates should be aware of recent milestones: in 2023, global targets were set to cut deaths from bacterial AMR by 10% by 2030 (baseline 2019). International cooperation also involves other agencies – for instance, the FAO and WOAH (World Organisation for Animal Health) support AMR efforts in farming. The UN's Sustainable Development Goals include targets for combating AMR (SDG 3.d). At this session, consider policies that could strengthen global coordination. Ideas might include establishing an international fund for AMR research, mandatory reporting of antibiotic use, trade rules to prevent dumping of antibiotics, or technology transfer to improve labs in poorer countries.

COMMITTEE BACKGROUND

The World Health Organization (WHO) is the UN's specialized agency for health, with a mandate to combat epidemics, set health standards and support global health initiatives. Founded in 1948, WHO now comprises 194 Member States. It is governed by the World Health Assembly (WHA), where all members meet annually to set policy. The WHA elects a 34-member Executive Board that implements decisions. As a WHO committee session, delegates here simulate an expert meeting on public health policy, though for MUN purposes it parallels a WHO Commission or high-level panel.

WHO's structure is relevant to your work. The WHA could be the forum for formal resolutions, but in practice it commissions expert bodies. For example, WHO has convened intergovernmental groups on AMR and coordinates the "Global Antimicrobial Resistance Surveillance System" (GLASS) to share data. Itssecretariat (the Director-General and technical staff) provides guidance and reports. Delegates should know that WHO outputs include guidelines (often non-binding) and "resolutions" from the Health Assembly, which are recommendations to members.

On AMR specifically, WHO leads the global AMR surveillance network and has published the "Global Report on Surveillance" and "Global Action Plan." Member States report to WHO on antibiotic consumption and resistance. Other UN bodies (UNEP, WHO, FAO joint program on AMR) also contribute. For this committee, assume WHO can recommend standards and encourage implementation but cannot enforce. The strength of your proposals will depend on political will and possible linkages to funding (e.g. through World Bank or UN development programs).

Historically, the WHA has passed resolutions urging national action on AMR (e.g. Resolution WHA68.7 in 2015). It's important to understand that WHO encourages multisector plans. Delegates should balance public health goals with feasibility: for instance, regulating antibiotic sales, improving veterinary oversight, or investing in clean water.

COMMITTEE STRUCTURE

The WHO is governed by its World Health Assembly (WHA), the main decision-making body composed of all 194 Member States. The WHA convenes yearly in Geneva. Our Model WHO committee can be seen as a WHA session on AMR. The WHO Executive Board (34 members) assists in setting the agenda, but for debate we assume open-floor discussion among all delegations. Delegates speak by country delegation, and chairs may be WHO representatives (the "bureau") rather than elected officials.

Often, WHO committees work by forming small working groups. For AMR, there might be sub-groups on human health policy, animal health, and cross-sectoral issues (e.g. R&D incentives, awareness campaigns). Negotiations are typically consensus-seeking; many WHO resolutions are adopted by acclamation. Draft texts should focus on concrete, science-based recommendations. Proposals might include establishing new WHO programs, coordinating with the Food and Agriculture Organization, funding for WHO-led laboratories, or setting global targets (e.g. limits on antibiotic use per livestock unit). As delegates prepare, consider quoting WHO sources (fact sheets, reports) and best-practice examples (some countries have enacted strict antibiotic laws or begun recycling older antibiotics from returned prescriptions). The WHO has many technical publications, but in debate you will mostly draft resolutions and engage in moderated caucuses. Remember to stress the global urgency: an integrated AMR strategy is crucial for the health and economies of all nations.